

KDIGO STAGE 2 AKI CRITERIA

In this synthetic restatement, stage two acute kidney injury is defined as a serum creatinine increase of two to three times baseline, or a urine output below 0.5 milliliters per kilogram per hour for at least twelve hours. Stage one corresponds to a creatinine rise of 1.5 to 1.9 times baseline or an absolute increase of 0.3 milligrams per deciliter within forty-eight hours.

AKI WORKUP AND MONITORING

Initial evaluation should include review of recent medications for nephrotoxic exposure, volume status assessment, and urinalysis with microscopy. Renal ultrasound is suggested when post-renal obstruction is plausible. Serum creatinine should be trended every twelve to twenty-four hours during active injury.

Avoidance of further nephrotoxins, including non-steroidal anti-inflammatory agents and intravenous iodinated contrast where feasible, is suggested through the recovery window.

RENAL REPLACEMENT TIMING

Synthetic guidance does not endorse uniformly early initiation of renal replacement therapy in stage two or three acute kidney injury without a compelling indication such as refractory hyperkalemia, acidosis, uremic complications, or volume overload unresponsive to diuretics.